

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Metronidazole (oral or vaginal) for the treatment of Bacterial Vaginosis

Summary of NICE / NICE CKS Guidelines for Bacterial Vaginosis

Overview

Bacterial vaginosis (BV) is the most common cause of abnormal vaginal discharge in women of reproductive age. It results from an overgrowth of normal vaginal flora with anaerobic bacteria, disrupting the normal lactobacillus-dominated microbiome.

Assessment

- **Clinical presentation:** Thin grey-white homogeneous vaginal discharge with a characteristic fishy odour, particularly noticeable after sexual intercourse.
- **Diagnostic criteria (Amsel criteria):** Requires 3 of 4: thin discharge, vaginal pH > 4.5, positive whiff test, clue cells on microscopy.

Management

- **First-line treatment:** Oral metronidazole 400mg twice daily for 5-7 days OR topical metronidazole 0.75% vaginal gel 5g once daily for 5 nights (equivalent efficacy).

Red flags requiring specialist referral

- **Pregnancy:** BV in pregnancy increases risks of preterm labour, premature rupture of membranes, and postpartum endometritis. Requires treatment, preferably in second/third trimester.
- **Signs of pelvic inflammatory disease (PID):** Lower abdominal pain, pelvic tenderness, fever - refer urgently to GP.
- **Concurrent sexually transmitted infections:** Require concurrent investigation and treatment.

Patient Group Direction

for the supply/administration of

Metronidazole 400mg tablets (oral)

for the treatment of

Bacterial Vaginosis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC

Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of bacterial vaginosis in non-pregnant women aged 16-65 years
Inclusion criteria	Women aged 16-65 years Clinical diagnosis of bacterial vaginosis (confirmed or presumptive on clinical grounds) Able to swallow tablets No contraindications
Exclusion criteria	Known hypersensitivity to metronidazole or nitroimidazoles Current alcohol consumption (risk of disulfiram-like reaction) Concurrent lithium therapy (increased lithium levels, risk of toxicity) Concurrent disulfiram therapy Active CNS disease or blood dyscrasia
Cautions	Hepatic impairment (mild to moderate) - adjust dose or frequency Renal impairment - may require dose reduction Pregnancy (avoid high-dose regimen in first trimester; seek GP guidance) Breastfeeding (significant amounts in breast milk; consider alternatives or temporary cessation) Drug interactions: warfarin (increased anticoagulant effect), phenytoin (increased phenytoin levels), lithium (increased lithium levels) Disulfiram-like reaction if alcohol consumed during or within 48 hours of treatment

Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.
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Details of the medicine

Name, form and strength of medicine	Metronidazole 400mg tablets
Legal category	POM
Storage	Store below 25°C in a dry place. Keep out of reach of children.
Route/method of administration	Oral, swallowed with water
Dose and frequency	400mg twice daily for 5-7 days (preferred regimen) Alternative: 2g as a single oral dose (less effective than 5-7 day course)
Quantity to be administered and/or supplied	10-14 tablets (400mg each)
Maximum or minimum treatment period	5-7 days for standard course; single dose alternative available
Adverse effects	Common: nausea, vomiting, metallic taste, abdominal discomfort, diarrhoea, headache, dark urine (may stain urine brown/black) Uncommon: rash, pruritus, constipation, dizziness Rare: peripheral neuropathy (with prolonged use > 8 weeks), hepatitis, anaphylaxis

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.

	A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	If taking oral metronidazole: Avoid all alcohol during the course of treatment and for 48 hours after the last dose. Alcohol can cause a disulfiram-like reaction (flushing, nausea, vomiting, abdominal pain, headache). Complete the full course of treatment as prescribed, even if symptoms resolve. Bacterial vaginosis is not a sexually transmitted infection, but your sexual partner(s) may not require treatment unless they develop symptoms. BV may recur; if symptoms return within 3 months, contact your GP for reassessment. If using the vaginal gel: You may use the applicator for insertion, but note that it may damage latex condoms and diaphragms - use alternative contraception. Report any adverse effects to your healthcare provider or via the Yellow Card scheme. Seek medical advice if symptoms do not resolve within 5-7 days of completing treatment, or if new symptoms develop (pelvic pain, fever).

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Bacterial vaginosis
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction
for the supply/administration of
Metronidazole 0.75% vaginal gel (Zidoval)
for the treatment of
Bacterial Vaginosis

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Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
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PGD Indication	Treatment of bacterial vaginosis in women aged 16-65 years
Inclusion criteria	Women aged 16-65 years Clinical diagnosis of bacterial vaginosis (confirmed or presumptive on clinical grounds) Able to insert gel intravaginally No contraindications
Exclusion criteria	Known hypersensitivity to metronidazole or nitroimidazoles
Cautions	May weaken latex condoms and diaphragms - advise alternative contraception during treatment and for 5 days after Pregnancy (safe to use; however, confirm diagnosis and consider

	referral to GP) Breastfeeding (minimal systemic absorption, but caution advised)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Metronidazole 0.75% vaginal gel (Zidoval)
Legal category	POM
Storage	Store below 25°C in a dry place. Keep out of reach of children.
Route/method of administration	Intravaginal application using applicator at bedtime
Dose and frequency	5g applicator inserted intravaginally once daily at bedtime for 5 nights
Quantity to be administered and/or supplied	1 tube containing 5g applicators (40g = 8 applications)
Maximum or minimum treatment period	5 nights of treatment
Adverse effects	Common: local vaginal irritation, vaginal burning, vaginal erythema Uncommon: secondary vaginal candidiasis, vulval pruritus Rare: systemic absorption effects (minimal due to topical application)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.

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